

إسم المريض رابعياً : ليلي عبدالمقصود عبدربه السيد

المحافظة : القاهرة

العنوان : ----

رقم البطاقة : 26409131100284 رقم التليفون : 01125658425

تاريخ تسجيل الإستمارة : 28-09-2026

العمر(تاريخ الميلاد) : 13-09-1964 المهنة : حاصل على دبلوم

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالغشاء البللوري

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالغشاء البللوري

خطة العلاج : أورام كيمائي علاج تدعيمى وتلطيفى [6387] قسم العلاج : الاورام

التعليق على التشخيص : -----

الاسم: د/محمد كيلاي
 الاسم: د/ندي حسن
 الاسم: د/نرمين رمضان
 الوظيفة: _____
 الوظيفة: _____
 الوظيفة: _____
 التوقيع: _____
 التوقيع: _____
 التوقيع: _____
 تعتمد صحة البيانات الخاصة بالمجالس
 رئيس اللجنة ومدير
 المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

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Name: ليلى عبدالمقصود عبدربه السيد National ID: 26409131100284	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

A patient of pleural mesothelioma for Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

لولى
عبدالمقصود عبدربه السيد
٢٠ ش لمعى المطيعى
المعصره - القاهره

١٩٦٤/٠٩/٢٢ ٢٦٤٠٩١٣١١٠٠٢٨٤

IS5366482

DM5
مقطوعه

٢٠٢١/٠١ ٢٦٤٠٩١٣١١٠٠٢٨٤
حاصلة على دبلوم فنى تجارى

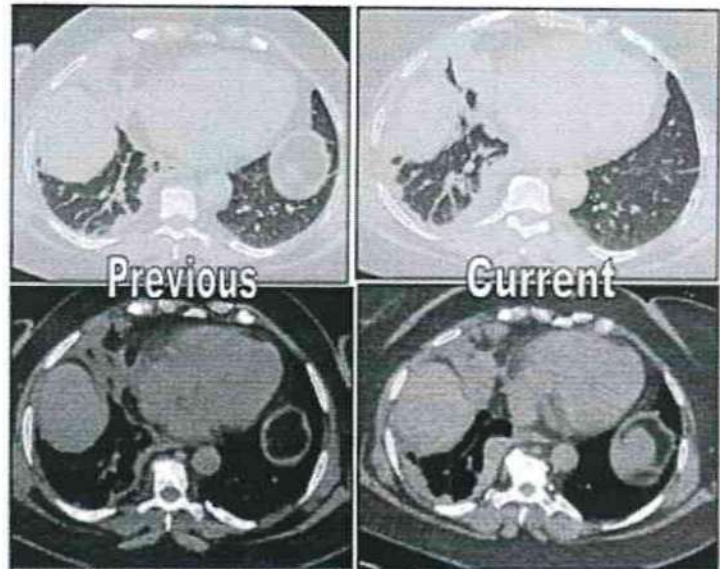
انثى مسلمة متزوجة
احمد ابراهيم على ابراهيم
البطاقة سارية حتى ٢٠٢٨/٠١/١٥

١

Multislice non-contrast CT scan of the chest with high resolution technique and multiplanar reformatting revealed:

**** Compared to the previous study dated 20/09/2025, the current study shows:**

- **Progressive course regarding the previously noted:**
 - Right mild pleural effusion with fissure extension.
 - Irregular circumferential costal, mediastinal and fissural as well as costophrenic recesses measuring maximally 3.0 cm.
 - Right paratracheal, retrotracheal, prevascular and posterior mediastinal lymph nodes the largest is seen in the right paratracheal group currently measuring about 1.7 x 2 compared to the previously measured of about 1.9x1.2 cm.
 - Right epiphrenic lymph node currently measuring about 2.2 x1.4 compared to the previously measured of about 1.5 x 1.1 cm.
- Newly noted multiple enlarged right internal mammary lymph nodes largest measures 1.7 x1.5 cm
- **Still noted:**
 - Subsegmental atelectasis the medial segment of middle and right lower lobes associated with right upper, middle and bilateral lobar parenchymal bands.



MOHANDSSIN : Safa Medical Tower - Hegaz Sq.
 HELIOPOLES : 73 Merghany St.
 MAADI : 150 Kornish El Nile- El Saudi Co. Bld.
 OCTOBER : Sheikh Zayed, Zayed 2000, Arkan Mall.
 NEW CAIRO : Concord Mall - 90 Southern St., 5th Settlement.
 NASR CITY : 68 Makram Ebeid St.

الفاسكان

Hotline: 16171

- Bilateral small pulmonary nodules with the mild size progression of the largest seen in the right lower lobe currently measuring about 1.2 cm compared to the previously measured of 0.9 cm in maximum axial dimension.
- **Still noted** bilateral costal and left mediastinal calcified pleural plaques.
- No pulmonary consolidative patches, cavitary lesions or bronchiectatic changes.
- No left pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Scanned upper abdominal cuts show hepatomegaly.

OPINION:

Follow-up study showing:

- Progressive course regarding the previously noted:
 - Circumferential irregular right pleural thickening.
 - Mediastinal lymph nodes.
 - Bilateral small pulmonary nodules with the further size progression of the largest one seen in the right lower lobe.
 - Newly developed multiple right internal mammary lymph nodes.
- ** For clinical correlation and further assessment to rule out underlying neoplastic pleural lesion.***

Dr. Mohamed A. Moneim, MD

Dr. Marwa Mohsen

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : ليلى عبد المقصود عبد ربه السيد

Analysis ID : 1052720

Age : 61 Y : 7 M : 1 D

Gender : Female

Conf. Code : 3615

Referred By Prof. Dr. : احمد الحفاوى

Clinical Diagnosis : Right anterior pleural thickening.

Nature of Specimen : Unstained slides (U/S guided core biopsy).

Receiving Date : 2026-04-28

Patient ID : 714943

PATHOLOGY REPORT

Gross :

Two referred slides, labelled as (411-26).

Microscopic :

Examination of the slides received revealed pleura tissue extensively infiltrated by tumor tissue formed of groups of malignant epithelial cells showing moderate nuclear pleomorphism and hyperchromasia, arranged in distorted acini, solid sheets and nests. There is marked desmoplasia and no necrosis.

Diagnosis :

Right anterior pleural thickening, Referred slides (U/S guided core biopsy),
EPITHELIAL TYPE NEOPLASM INFILTRATING PLEURA.

- CASE FOR IMMUNOPHENOTYPING.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

Shady Elia

Elia anis



This report is electronically signed , any abrasion or modification would render it void .

1/1

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٠١٢٨٧٠٠٥٢١٧ - ٠١٢١٢٩٤٥١١٥

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Patient Name : ليلى عبد المقصود عبد ربه السيد

Analysis ID : 1056830

Age : 61 Y : 7 M : 2 D

Gender : Female

Conf. Code : 7801

Referred By Prof. Dr. : احمد الحفاوى

Clinical Diagnosis : Right anterior pleural thickening

Nature of Specimen : Referred block [labelled as 411-26]

Receiving Date : 2026-05-11

Patient ID : 714943

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain.

The sections were treated against:

- CEA \ Calretinin \ D2-40 \ MOC 31

Results :

Tumor cells are NEGATIVE for CEA.

Tumor cells are POSITIVE for Calretinin.

Tumor cells are focally POSITIVE for D2-40.

Tumor cells are NEGATIVE for MOC 31.

Conclusion :

Referred block, FINDINGS ARE COMPATIBLE WITH EPITHELIOID MESOTHELIOMA.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

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عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٠١٢١٢٩٤٥١١٥ - ٠١٢٨٧٠٠٥٢١٧

Dr. saad S. Eissa, M. D.

Dd' Etat C E S Path (Paris), D.A.E. (Paris)
Fellow Johns Hopkins Medical School (USA)

Dr. Mohab S. Eissa, M. D.

M.D. Pathology Oncology
Pathology Consultant

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دكتور مهاب سعد عيسى

دكتورة باثولوجيا الاورام
إستشارى باثولوجيا

PATHOLOGY REPORT

Name: LAILA ABDEL MAKSOUD ABDO RABO

Age: 62 years

Clinical picture: Right Pleura, Extension Irregular Sheets and Thickening (Cosal, Mediastinal, Diaphragmatic) Mediastinal and Hilar Lymph Nodes

Nature of specimen: Right Anterior Pleural Thickening US Guided Core Biopsy

Referred by: Prof. Dr. AHMED EL HALAFAWY/MOHAMED GAMAL/MRC

Date: 20 / 4 / 2026

Gross:

Core biopsy composed of tiny fragments of grayish white tissues embedded as such.

نيلى عبد المقصود عبد ربه السيد



1052720

2026-04-28

د/ احمد الحلفاوى

Microscopic:

Sections revealed malignant biphasic tumor formed of clusters of malignant epithelial cells and spindle shaped cells extensively infiltrated and replaced the pleura.

Diagnosis:

RIGHT ANTERIOR PLEURAL THICKENING US GUIDED CORE BIOPSY: SUGGESTIVE OF MALIGNANT MESOTHELIOMA VERSUS METASTATIC ADENOCARCINOMA. IMMUNOPHENOTYPING -AVAILABLE IN OUR LAB- (CK, CALRETININ, CK5/6 AND GATA-3) IS REQUIRED.

(Code: 411 / 4 / 2026)

Date of issue: 25 / 4 / 2026

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دكتوراة باثولوجيا الاورام
إستشارى باثولوجيا

PATHOLOGY REPORT

Name: LAILA ABDEL MAKSOUD ABDO RABO

Age: 62 years

Clinical picture: Right Pleura, Extension Irregular Sheets and Thickening (Cosal, Mediastinal, Diaphragmatic) Mediastinal and Hilar Lymph Nodes

Nature of specimen: Anterior Pleural Thickening US Guided Core Biopsy and Immunophenotyping

Referred by: Prof. Dr. AHMED EL HALAFAWY/MOHAMED GAMAL/MRC

Date: 3 / 5 / 2026

Gross:

Core biopsy composed of tiny fragments of grayish white tissues embedded as such. Unstained sections stained with CK, CALRETININ, CK5/6 and GATA-3 monoclonal antibodies using benchmark automatic stainer (Ventana USA), DAB as substrate and hematoxylin as counter stain.

Microscopic:

Sections revealed malignant biphasic tumor formed of clusters of malignant epithelial cells and spindle shaped cells extensively infiltrated and replaced the pleura. Tumor cells are positive for CK, GATA-3 and CALRETININ, negative for CK5/6 monoclonal antibodies confirming.

Diagnosis:

RIGHT ANTERIOR PLEURAL THICKENING US GUIDED CORE BIOPSY AND IMMUNOPHENOTYPING: CONSISTENT WITH METASTATIC ADENOCARCINOMA, MOST LIKELY OF PRIMARY BREAST. TUMOR CELLS ARE POSITIVE FOR CK, GATA-3 AND CALRETININ, NEGATIVE FOR CK5/6.

(Code: 52 / 5 / 2026)

Date of issue: 10 / 5 / 2026

معمل: ٢٨١٤٠١٨ - ٢٤٥٠٥٧٢٤
مكتب: ٢٨١٠٨١٠٥

معمل ١٢٠٦ بطرس غالى منفرع من إبراهيم اللقاني ميدان روكسى - مصر الجديدة - الدور الخامس

E-Mail : drsaadeissa@hotmail.com

Name: *Laila Abd Al Maksoud Abd Rabbo* ID: 4385990
 Referred by: *Prof. Ahmed Al Haffawi* Date: 15/04/2026

¹⁸F-FDG PET/CT examination

Clinical Indication:

- Female patient with history of right pleural thickening, for assessment.

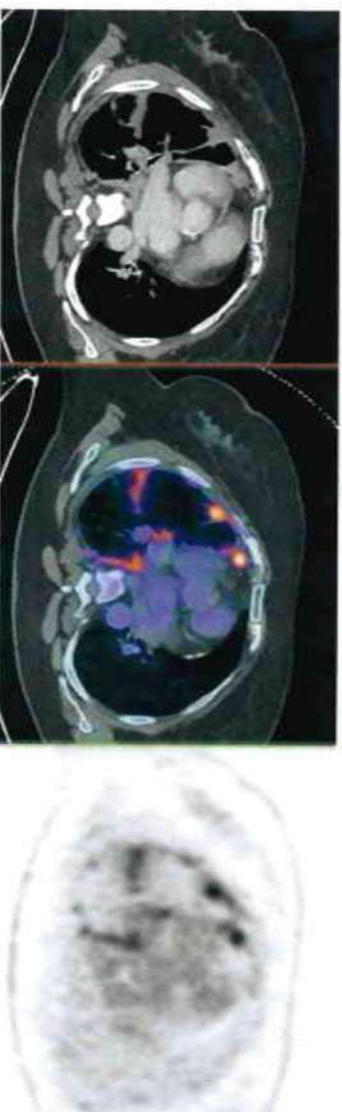
Administered activity: 388.5 MBq. ¹⁸F-FDG B.G.L. : 103 mg/dl

Examination protocol:

- A Whole body PET/CT study was obtained from skull vertex to the mid thigh using a low dose non contrast CT for attenuation correction
- Diagnostic multislice (64 channels) CT examination after IV contrast administration of the same region.

I. PET/CT Findings:

- Right pleura:



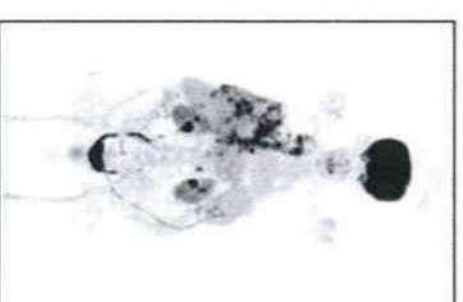
- Increased FDG activity corresponding to irregular sheet like and nodular lesions involving the costal, mediastinal, fissural and diaphragmatic reflections as well as the pleural recess with SUVmax 15.2 in costal nodular lesion measures about 2.8 cm.
- No significant pleural effusion.
- **Left pleural:** no FDG avid lesions or effusion.

• Lymph nodes:

- FDG avid mediastinal right internal mammary, right para-tracheal, right tracheo-bronchial, epiphrenic, cardiophrenic, right retro-crural and posterior mediastinal LNs with SUVmax 15.8 and measures 2.0 cm in right trache-bronchial LN.



- FDG avid abdominal celiac LN with SUVmax 10.2 and measures 1.0 cm.
- **Liver:** enlarged with no FDG avid focal lesions.
- **Lungs:**
 - Tiny non FDG avid pulmonary nodules, for follow-up.
 - Mosaic attenuation of lung fields.
 - right basal atelectasis.



- **Bone:**
 - No FDG avid lytic or sclerotic lesions.
 - FDG activity related to shoulders, inflammatory.
 - Degenerative changes of the spine.
- FDG activity within bilateral gluteal soft tissue thickening, inflammatory.

II. Diagnostic CT Findings:

- No pericardial or peritoneal collections.
- The spleen, pancreas, adrenals, kidneys, pelvic organs, aorta and IVC appear unremarkable.

OPINION:

- Metabolically active right pleural lesions, likely neoplastic with metastatic lymph nodal deposits as described, for histopathology.

Radiologist
Dr. Mohamed Hoseny, MD

Nuclear Medicine Physicians
Dr. Ahmed Tawakol, MD
Dr. Ahmed Badawy, MD

